

CITYCARE GENERAL HOSPITAL

HOSPITAL STANDARD OPERATING PROCEDURE

General Operations, Governance & Quality Framework

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TABLE OF CONTENTS

1	Purpose & Scope
2	Definitions & Abbreviations
3	Roles & Responsibilities
4	Hospital-Wide Operational Workflow
5	Patient Rights & Consent Framework
6	Infection Prevention & Control
7	Documentation & Record Management
8	Incident Reporting & Risk Management
9	Staff Training & Competency
10	Exceptions, Non-Conformances & Review

1. PURPOSE & SCOPE

This Standard Operating Procedure (SOP) establishes the overarching operational, clinical governance, and quality management framework for CityHospital. It defines the minimum standards applicable to all departments, units, and staff categories to ensure patient safety, regulatory compliance, and continuous service excellence.

1.1 Objectives

- Provide a unified framework governing all hospital operations.
- Ensure consistent delivery of safe, effective, and patient-centred care.
- Establish clear lines of accountability at all organisational levels.
- Facilitate compliance with national healthcare regulations and accreditation standards (NABH/JCI).
- Promote a culture of continuous quality improvement and evidence-based practice.

1.2 Scope

This SOP applies to all permanent, temporary, and contractual staff including physicians, nurses, allied health professionals, administrative personnel, housekeeping, security, and third-party service providers operating within the hospital premises.

NOTE: Department-specific SOPs supplement this general framework and must be read in conjunction with this document.

2. DEFINITIONS & ABBREVIATIONS

Term / Abbreviation	Full Definition
CMO	Chief Medical Officer
CNO	Chief Nursing Officer
QA	Quality Assurance
IPC	Infection Prevention and Control
EMR	Electronic Medical Record
SOP	Standard Operating Procedure
JCI	Joint Commission International
NABH	National Accreditation Board for Hospitals
RCA	Root Cause Analysis
PPE	Personal Protective Equipment
MRN	Medical Record Number
ICU	Intensive Care Unit
OPD	Outpatient Department
IPD	Inpatient Department
CCTV	Closed Circuit Television
CAPA	Corrective and Preventive Action

3. ROLES & RESPONSIBILITIES

Role	Department	Key Responsibilities
Chief Medical Officer	Executive	Overall clinical governance, SOP approval, regulatory liaison
Chief Nursing Officer	Nursing	Nursing standards, staff deployment, care quality oversight
Department Heads	All Depts.	Departmental SOP adherence, staff management, reporting
Quality Manager	QA Department	SOP audits, accreditation, incident analysis, CAPA tracking
Ward In-charge Nurse	All Wards	Daily safety rounds, staff briefings, documentation oversight
IPC Nurse	IPC Team	Infection surveillance, PPE compliance, outbreak management
Medical Records Officer	Medical Records	EMR integrity, record retention, confidentiality
HR Manager	Human Resources	Staff onboarding, training compliance, credential verification

4. HOSPITAL-WIDE OPERATIONAL WORKFLOW

4.1 Daily Operations Cycle

- Step 1.** 0600 hrs — Night shift supervisor submits handover report to incoming morning supervisor; all critical incidents flagged.
- Step 2.** 0700 hrs — Department heads conduct unit walkrounds; bed status and staffing levels confirmed in HIS.
- Step 3.** 0800 hrs — Morning clinical briefing: admissions, discharges, surgeries, and high-dependency patients reviewed.
- Step 4.** 0900 hrs — Pharmacy, lab, and radiology daily workload plans activated; consumable stock checked.
- Step 5.** Throughout Day — Patient care activities proceed per individual departmental SOPs; EMR updated in real time.
- Step 6.** 1400 hrs — Afternoon supervisor walkround; patient safety checks conducted; visitor policy enforced.
- Step 7.** 2000 hrs — Evening handover: outstanding investigations, critical results, and overnight watch-list communicated.
- Step 8.** 2200 hrs — Night safety rounds conducted by on-call physician and senior nurse.

4.2 Interdepartmental Communication Protocol

- All critical lab values must be telephoned to the treating physician within 15 minutes of verification.
- Referrals between departments must be documented in the EMR and acknowledged within 30 minutes.
- Patient transfer between wards requires written transfer order and two-nurse verification.
- Radiology results flagged as urgent must trigger immediate attending physician notification.
- All verbal orders must be read back and confirmed; documented in EMR within 1 hour.

5. PATIENT RIGHTS & CONSENT FRAMEWORK

Every patient has the right to dignified, respectful, and culturally sensitive care. CityHospital upholds the following patient rights as mandated by national law and international accreditation standards.

5.1 Core Patient Rights

- Right to be informed about diagnosis, treatment plan, and alternatives in a language they understand.
- Right to provide or withhold informed consent before any procedure.
- Right to confidentiality of personal and medical information.
- Right to a second opinion without prejudice to ongoing care.
- Right to access their own medical records within 48 hours of written request.
- Right to lodge grievances and receive a response within 7 working days.
- Right to pain management and palliative care where applicable.

5.2 Consent Workflow

Step 1. Physician explains procedure, risks, benefits, and alternatives in the patient's preferred language.

Step 2. Patient or authorised guardian reviews and signs the consent form; witness signs alongside.

Step 3. Signed consent placed in EMR immediately; original retained in physical file.

Step 4. For emergencies where the patient is unable to consent, two-physician endorsement is required and documented.

WARNING: Consent must be obtained BEFORE any invasive procedure, sedation, or anaesthesia. No procedure may commence without documented consent or documented emergency override.

6. INFECTION PREVENTION & CONTROL

IPC is a mandatory component of all clinical activities. Non-compliance with IPC standards is a serious patient safety violation subject to disciplinary action.

6.1 Standard Precautions — All Staff

- Perform hand hygiene (WHO 5 Moments) before patient contact, before aseptic procedures, after body fluid exposure, after patient contact, and after touching patient surroundings.
- Don appropriate PPE (gloves, mask, gown, eye protection) based on the risk assessment for each task.
- Dispose of sharps immediately in designated puncture-resistant containers — never recap needles.
- Handle linen soiled with body fluids as infectious waste; use sealed double-bag method.
- Clean and disinfect patient equipment between uses with hospital-approved disinfectant.

6.2 Outbreak Response Protocol

Step 1. IPC nurse notified of suspected outbreak cluster; preliminary assessment within 1 hour.

Step 2. Affected ward placed under enhanced surveillance; cohorting of cases initiated immediately.

Step 3. Microbiological samples collected and lab processing prioritised.

Step 4. CMO and IPC Committee convened; escalation to public health authorities if threshold is met.

Step 5. Daily line listing maintained; decontamination schedule activated for all affected areas.

7. DOCUMENTATION & RECORD MANAGEMENT

7.1 EMR Standards

- All clinical entries must be time-stamped, author-attributed, and completed within 2 hours of the event.
- Abbreviations not on the hospital-approved list are prohibited in clinical documentation.
- Errors must be corrected using a single strike-through with initials and date — white-out is strictly prohibited.
- Discharge summaries must be completed within 24 hours of patient discharge.
- All verbal orders must be co-signed by the ordering physician within 24 hours.

7.2 Retention Schedule

Record Type	Retention Period	Storage Location
Adult inpatient medical records	10 years post last encounter	EMR + Physical archive
Paediatric records	Until age 25 or 8 yrs post death	EMR + Physical archive
Surgical/anaesthesia records	15 years	EMR
Incident reports	7 years	QA Department secure storage
Staff training records	Duration of employment + 5 yrs	HR System

8. INCIDENT REPORTING & RISK MANAGEMENT

8.1 Incident Classification

- **Level 1 (Near Miss):** Intercepted before reaching the patient. Report within 24 hours.
- **Level 2 (No Harm):** Reached patient but caused no detectable harm. Report within 12 hours.
- **Level 3 (Low Harm):** Minor injury, no lasting effects. Report within 6 hours.
- **Level 4 (Moderate/Severe Harm):** Significant patient harm. Report immediately; RCA mandatory.
- **Level 5 (Sentinel Event):** Death or permanent harm. Report within 1 hour; external notification required.

8.2 Reporting Workflow

Step 1. Staff identifies incident; ensures immediate patient safety first.

Step 2. Incident form completed on EMR within the mandated timeframe.

Step 3. Ward in-charge notified immediately; Department Head informed.

Step 4. QA team reviews report; classifies severity; assigns investigation lead.

Step 5. RCA completed within 5 business days; CAPA developed and implemented.

Step 6. Learnings communicated via safety bulletin to all relevant staff within 2 weeks.

9. STAFF TRAINING & COMPETENCY

- All new staff must complete hospital induction within 7 days of joining, covering IPC, fire safety, patient rights, EMR use, and emergency codes.
- Annual mandatory training for all staff: CPR/BLS, fire evacuation, IPC refresher, manual handling.
- Clinical staff must demonstrate annual competency assessment for their scope of practice.
- Training records are maintained in the HR system; non-compliance triggers escalation to Department Head.
- Preceptorship programme: all newly qualified staff assigned a senior mentor for a minimum 3-month period.
- Simulation-based training conducted bi-annually for high-risk scenarios (cardiac arrest, fire, mass casualty).

NOTE: Training non-compliance will be treated as a performance issue and addressed through the hospital disciplinary framework.

10. EXCEPTIONS, NON-CONFORMANCES & REVIEW

10.1 Managing Exceptions

- Any deviation from this SOP must be documented in the EMR with clear clinical justification.
- Exceptions require counter-signature by the relevant Department Head.
- Repeated exceptions by an individual trigger a formal performance review.
- Repeated systemic exceptions trigger SOP revision review by the Clinical Governance Committee.

10.2 SOP Review Cycle

- This SOP is reviewed annually or earlier if triggered by a sentinel event, regulatory change, or clinical evidence update.
- All proposed amendments submitted to the Clinical Governance Committee for approval.
- Approved changes notified to staff via the intranet and team briefings within 7 days of ratification.
- Version history maintained; superseded versions archived in the document management system.